



WNHS PROCEDURE	
Physical Health Care of Mother and Baby Unit Inpatients	
Scope (Staff):	Mental health staff working in WNHS
Scope (Area):	Mother Baby Unit (MBU)

1. Aim

The purpose of this guideline is to provide direction and information to all staff of the Mother and Baby Unit (MBU) regarding the physical health of inpatients that is consistent with the [Mental Health Act 2014](#) (MHA 2014) (Part 15, Division 1 s241) and [Chief Psychiatrist's Standards for Clinical Care: Physical Health Care of Mental Health Consumers](#).

2. Risk

Non-compliance with this policy will:

Breach legislative requirements	☒	Impact on Patient Quality of Care	☒
Breach National/State/Mandatory Policy	☒	Impact on Patient Safety	☒
Breach professional standards	☐	Impact financial operations	☐
Damage NMHS reputation	☐	Impact on Staff Safety	☐
Other	☐		

3. Key Points

All patients admitted to MBU will have their physical health assessed, monitored and managed as required. In addition staff will ensure:

- the role of the partner/support person is recognised in the assessment and management of physical health needs
- appropriate consent is obtained prior to physical examination of the patient. Although consent is not required from involuntary patients, good practice dictates that this should be sought
- the medical practitioner conducting the physical examination contacts the patient's General Practitioner (GP) and/or specialist if any clarification regarding medical issues is required.

4. Process

All physical health assessments will include the following:

- medical history
- obstetric history
- physical examination
- specific screening processes for medical issues that may contribute to psychiatric presentation and common physical co-morbidities based on the patient's demographic, mental illness profile and known treatment side effects, including review of risks of metabolic syndrome.
- consideration of falls risk and any requirements for further assessment
- noting of any acute or chronic general medical treatment or management.

Admission Procedure

- A brief health check will be completed to ensure the patient is physically well – temperature, pulse, respirations and blood pressure and any current screening questions are asked
- All patients will have their medical history recorded and a physical examination conducted within 12 hours of admission to the MBU.
- If a physical examination cannot be performed on admission or within 12 hours, the reasons for this must be clearly documented in the healthcare record and followed up with the treating team.
- If a patient actively refuses or is unable to co-operate the medical practitioner must make other attempts at reasonable intervals thereafter
- All investigations and findings are clearly documented in the SSCD Mental Health Physical Examination ([SMHMR 903](#))

Nursing staff are responsible for the following aspects of the physical examination:

- recording temperature, pulse, respiration rate, blood pressure as well as height, weight, body mass index and physical appearance as per the State-wide Standardised Clinical Documentation ([SMHMR903](#)).
- electrocardiogram is to be performed on admission. It may be repeated during admission and at discharge if indicated.
- physical observations are, at a minimum, routinely performed daily on each patient. Clinical concerns may indicate more frequent physical observation
- All staff must escalate concerns as per [WNHS Recognising and Responding to Acute Clinical Deterioration \(Physiological and Mental Health\) Policy](#).
- Patients have their weights measured weekly, at a minimum, and documented in the patient healthcare record.

The medical practitioner is responsible for a comprehensive physical systems examination. This will include a discussion about contraception. Depending on a number of factors such as physical health and medication prescribed, the medical practitioner will consider a range of further investigations. All abnormal results will be followed up for appropriate treatment by the Medical Officer and referred to specialist services as required.

- All patients admitted to the MBU are required to have a falls risk assessment within 6 hours of admission using the North Metropolitan Area Health Service Mental Health Falls Risk Management Tool (FRMT) PMR 100.
- All patients are assessed for risk of pressure injuries as part of a full physical examination and, if indicated, a formal assessment completed.
- The Fagerstrom assessment will be completed on all patients who smoke and/or vape and a management plan reflecting their smoking requirements developed. The MBU (as is consistent with other NMHS sites) is a smoke-free site and will follow the [WNHS Guideline- Management of Nicotine Dependence](#).

Management of Acute Physical Illness

- The MBU does not have capacity to provide acute medical management for patients with significant physical illness.
- Patients with respiratory illness will be managed according to [the WNHS Infection Prevention and Management Respiratory Virus Infection Policy](#). Patients (and their babies) with suspected infectious illness must be managed according to Infection Prevention and Management Guidelines ([WNHS Infection Prevention Manual](#))
- Any acute physically unwell patients will be stabilised as far as appropriate and then be transferred to either SCGH Emergency Department or another suitable physical health facility, with appropriate handover documentation [MBU Clinical Handover procedure](#)
- Consideration of appropriate transportation will be made by the treating team at the time.

Management of ongoing physical health conditions

- Nursing staff will support patients to find a local GP if they are unable to identify one prior to discharge.
- The medical officer will handover to the nominated GP all relevant physical care information at the time of discharge, highlighting any areas that require follow up in the discharge summary. This will also include an ongoing medication plan.
- The discharge summary can be sent to other relevant specialists with the patient's consent.

Monitoring and Evaluation

- Auditing of medical record – physical examination documented with 12 hours of admission and vitals completed as per medical requirements.
- Evidence of physical health care components in the Treatment, Support and Discharge Plans.

Related internal policies, procedures and guidelines

[Clinical Handover - MBU](#)

[Women's and Perinatal Mental Health Referral and Management Guideline](#)

[Chief Psychiatrist's Standards for Clinical Care: Physical Health Care of Mental Health Consumers](#)

[WNHS Recognising and Responding to Acute Clinical Deterioration \(Physiological and Mental Health\) Policy](#)

[WNHS Guideline- Management of Nicotine Dependence](#)

[WNHS Infection Prevention and Management Manual](#)

References

[Mental Health Act 2014](#)

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Policy Sponsor	Head of Department, Mother Baby Unit/ Director of Psychiatry				
Policy Author	Coordinator of Nursing – Mental Health				
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Approved by:	MBU Team Meeting			Date:	January 2025
Endorsed by:	Mental Health Clinical Practice and Outcomes Committee			Date:	03/02/2025
NSQHS Standards Applicable:	 Std 1: Clinical Governance  Std 5: Comprehensive Care				
National Standards for Mental Health Services	☐ Std 1: Rights and Responsibilities ☐ Std 2: Safety ☐ Std 3: Consumer and Carer Participation ☐ Std 4: Diversity Responsibility ☐ Std 5: Promotion and Prevention  Std 6: Communicating for Safety  Std 8: Recognising and Responding to Acute Deterioration ☐ Std 9: Integration ☐ Std 10: Delivery of Care ☐ 10.1 Supporting Recovery ☐ 10.2 Access ☐ 10.3 Entry ☐ 10.4 Assessment and Review				

☐ Std 6: Consumers ☐ Std 7: Carers ☐ Std 8: Governance, leadership and management	☐ 10.5 Treatment and Support ☐ 10.6 Exit and Re-entry
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Version History:

Version Notes	Dates
First published version	May 2012
Regular review and update	August 2021 (other dates unknown)
Three yearly review – amendments include: Added: • Screening to include review of risks of metabolic syndrome • Weekly weights • Patients who smoke and/or vape and a link to WNHS Guideline Management of Nicotine Dependence • Discharge summary sent to other relevant specialists with the patient's consent. • WNHS Infection Prevention and Management Respiratory Virus Infection Policy Removed • Patient flow MBU Covid guideline removed.	February 2025

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